NEW SCRIPTS · VALUE PER SECOND

10 New Scripts + 1 Full-System Video

Built from your Content Bank for clinic-owner doctors: AI systems, personal branding and clinic branding, plus one hero video showing the whole journey from content strategy to auto-reply, booking, reminders, feedback and the returning patient.

Prepared for Dr. Azka · 1 November 2026

Overview

#	Category	Script	Stage	Length
AI-1	AI Systems	Watch an AI receptionist answer a clinic's call at 11 PM	BOFU	~45 s
AI-2	AI Systems	The first AI system every clinic should set up	MOFU	~35 s
AI-3	AI Systems	As a doctor, here's what I never let an AI receptionist say	MOFU	~40 s
AI-4	AI Systems	Your reminder message is why patients don't show up	MOFU	~40 s
PB-1	Personal Brand	'Dr. ___ is the doctor for ___'	MOFU	~40 s
PB-2	Personal Brand	Doctors: you already have 100 content ideas	TOFU	~35 s
PB-3	Personal Brand	Why your ChatGPT scripts sound like every other doctor	TOFU	~40 s
CB-1	Clinic Brand	Patients don't choose clinics. They choose doctors.	TOFU	~40 s
CB-2	Clinic Brand	Your marketing agency isn't medical. Ask them these 5 questions.	MOFU	~45 s
CB-3	Clinic Brand	The camera-shy doctor in your clinic can still be a star	TOFU	~40 s
FULL	Full System (Hero Video)	From one Reel to a returning patient: the complete system I build for clinics	BOFU	~2 min (Reel version ~90 s, also works for YouTube/LinkedIn)

Every script follows the same rules

A hook with the tension in the first 3 seconds · one idea per line · one core message doctors will repeat · a CTA that starts a conversation · on-screen text that adds structure, not subtitles.

Suggested posting order

Week 1: CB-1, PB-2, AI-2 · Week 2: PB-1, AI-3, CB-3 · Week 3: AI-4, PB-3, CB-2 · Week 4: AI-1, then **FULL** (pin it to your profile).

Demos and data

For every AI demo use a fictional clinic and patient (e.g. 'Glow Dermatology', 'Sara') and a demo number. Never show real patient messages or calls.

SCRIPT AI-1 · BOFU · ~45 S

Watch an AI receptionist answer a clinic's call at 11 PM

Format: Screen recording / phone-call demo on a DEMO clinic number + your face in a corner bubble.

Built from: Content Bank, Pillar 8.1 (AI receptionist & missed calls)
Core message: A patient who calls at 11 PM is ready to book. Someone, or something, should answer.
Value check: Clinic owners see a real AI call: what it says, when it books, and when it hands over to a human.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	It's 11 PM. A patient calls a dermatology clinic. Nobody's there. Watch what happens.	OST: "11:04 PM · Clinic closed" · Visual: phone ringing
3–14s	[Play demo audio] 'Hi, you've reached Glow Dermatology. I'm the clinic's virtual assistant. How can I help?' 'Hi, I wanted to ask about laser for pigmentation.'	Visual: live transcript on screen
14–24s	It asks two questions: is this your first visit, and what days suit you? Then it offers two real slots from the clinic's calendar.	OST: "2 questions → 2 slots"
24–32s	And if the patient asks a medical question, like 'is it safe in pregnancy?', it doesn't guess. It says a doctor will call them back, and flags it for the team.	OST: "Medical question → human" 🛑
32–40s	By the time the clinic opens, the patient is booked, and the team has a summary of the call.	Visual: booking confirmed + call summary
40–45s	Your clinic closes at 8. Your patients don't stop searching at 8.	OST: "Patients don't stop at 8 PM."

CTA: Clinic owners: want to hear this on your own clinic's number? DM me 'AI'.

Caption: An after-hours call is a ready-to-book patient. Here's what an AI receptionist built for clinics says, and what it never says.

Filming tip: Use a demo clinic name and your own voice as the 'patient'. Never use a real patient call.

SCRIPT AI-2 · MOFU · ~35 S

The first AI system every clinic should set up

Format: Talking head + simple phone mock-up showing the auto-text arriving.

Built from: Content Bank, Pillar 8.1 (missed-call text-back)
Core message: Start with the simplest system: never let a missed call disappear.
Value check: Clinic owners get one easy, low-cost system they can set up this month.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	If you set up only one AI system in your clinic this month, make it this one.	OST: “System #1”
3–9s	Missed-call text-back. Your team misses a call. Within seconds, the patient gets a text.	Visual: missed call → text arrives
9–15s	‘Sorry we missed your call, this is Smile Dental. How can we help? You can reply here.’	OST: the message on a phone screen
15–24s	Why it works: a patient who can't reach you usually calls the next clinic. A text in seconds keeps the conversation with you.	OST: “Missed call ≠ lost patient”
24–31s	Then your team, or your AI, replies, asks what they need, and offers two times.	OST: “Reply → question → 2 slots”
31–35s	No new staff. No big software change. Just no more lost calls.	OST: “No more lost calls.”

CTA: How many calls do you think your clinic misses each week? Guess in the comments.

Caption: The simplest AI system is often the most profitable one. Start here.

Filming tip: Film the phone mock-up with a real phone and a demo number. It looks more believable than a graphic.

SCRIPT AI-3 · MOFU · ~40 S

As a doctor, here's what I never let an AI receptionist say

Format: Talking head, serious tone. Red line graphic for each rule.

Built from: Content Bank, Pillar 8.1 & 8.4 (AI red lines, safe AI)
Core message: AI can handle the admin, but clinical judgment always stays with the doctor.
Value check: Doctors who are scared of AI get 4 clear safety rules, and see why a doctor should design clinic AI.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	I build AI systems for clinics, and as a doctor, there are four things I never let them say.	OST: "4 AI red lines"
3–10s	One: no diagnosis. If a patient describes a symptom, the AI doesn't name what it might be. It books them with a doctor.	OST: "1. No diagnosis"
10–17s	Two: no medical advice. 'Can I take this with my medication?' goes straight to a clinician.	OST: "2. No medical advice"
17–23s	Three: no promises. Never 'guaranteed results' or 'it won't hurt at all'.	OST: "3. No promises"
23–31s	Four: it never pretends to be human. It says it's the clinic's assistant, and hands over to a person whenever the patient is anxious, upset or urgent.	OST: "4. Always says it's AI"
31–40s	AI should make your clinic faster and kinder, never riskier. That's why a doctor should be involved in designing it.	OST: "Faster. Kinder. Never riskier."

CTA: What worries you most about using AI in your clinic? Tell me below. I'll answer in my next video.

Caption: Clinic AI needs clinical guardrails. Here are my 4 non-negotiables as a doctor.

Filming tip: Film this one slightly slower and calmer than usual. Trust content works best in a 'consultation voice'.

SCRIPT AI-4 · MOFU · ~40 S

Your reminder message is why patients don't show up

Format: Split screen: the weak reminder vs. the 3-message sequence on a phone.

Built from: Content Bank, Pillar 8.3 (no-shows and reminders)
Core message: No-shows are usually a system problem, not a patient problem.
Value check: Clinics get a 3-step reminder sequence they can copy today.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	If patients keep not showing up, look at your reminder message first.	OST: "No-shows? Check this first."
3–9s	Most clinics send one line: 'Reminder: appointment tomorrow, 3 PM.' Easy to ignore. Nothing to do.	Visual: weak reminder, crossed out
9–17s	Try three messages. Forty-eight hours before: confirm, give any prep instructions, and ask them to reply YES.	OST: "48h: confirm + prep + reply YES"
17–25s	Twenty-four hours before: a friendly reminder and a one-tap link to reschedule. A reschedule is better than an empty chair.	OST: "24h: reminder + easy reschedule"
25–32s	On the day: a short note with directions or parking.	OST: "Day-of: directions"
32–40s	All automated, in your clinic's voice. And track your no-show rate every month, because what gets measured gets fixed.	OST: "Measure your no-show rate"

CTA: Do you know your clinic's no-show rate? Yes or no in the comments.

Caption: One reminder line isn't a reminder system. Here's the 48h / 24h / day-of sequence I set up for clinics.

Filming tip: Screen-record the three messages arriving on a phone. It makes the sequence feel real.

SCRIPT PB-1 · MOFU · ~40 S

'Dr. ___ is the doctor for ___'

Format: Talking head + fill-in-the-blank text on screen.

Built from: Content Bank, Pillar 4.2 (positioning)
Core message: If patients can't say what you're known for, your brand isn't working yet.
Value check: Doctors get a one-sentence positioning exercise they can do in 5 minutes.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	If your patients can't finish this sentence about you, your personal brand isn't working.	OST: "Dr. ___ is the doctor for ___."
3–10s	'Dr. ___ is the doctor for ___.' One or two things you want to be known for in your city.	Visual: blanks pulsing
10–18s	Most doctors try to be known for everything. Acne, lasers, fillers, hair, all at once. Patients remember none of it.	OST: "Everything = nothing"
18–26s	You can still treat everything. Just be KNOWN for something. 'The dermatologist for melasma in Dubai.' 'The dentist for natural veneers in Miami.'	OST: examples appear
26–34s	Then every video you post reinforces that sentence. That's how a profile turns into a reputation.	OST: "Every post reinforces it"
34–40s	Positioning first. Content second.	OST: "Positioning first."

CTA: Fill in your own blank in the comments. I'll reply to as many as I can.

Caption: One sentence decides whether patients remember you. Fill in the blank.

Filming tip: The comment answers are perfect DM openers for outreach: reply, then message them.

SCRIPT PB-2 · TOFU · ~35 S

Doctors: you already have 100 content ideas

Format: Talking head + a notepad / phone notes visual filling with patient questions.

Built from: Content Bank, Pillar 1.2 (turning expertise into content)
Core message: Your clinic is your content library. Every question patients ask twice is a video.
Value check: Doctors get a simple system to never run out of ideas.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	Doctors, you'll never run out of content ideas again. You already hear them every day.	OST: "100 ideas you already have"
3–10s	Every question a patient asks you twice is a video. 'Does it hurt?' 'When will I see results?' 'Can I wear makeup after?'	Visual: questions typed into notes
10–17s	Ask your front desk to write down every patient question for two weeks.	OST: "2-week question log"
17–25s	That list is a month of content. Each question becomes a hook: 'The question every Botox patient is too shy to ask.'	OST: question → hook
25–35s	And answer it the way you do in consultation, not like a textbook. Patient's worry first, medicine second.	OST: "Worry first. Medicine second."

CTA: What's the one question your patients ask you most? Drop it below.

Caption: Content ideas aren't hard to find. They're walking into your clinic every day.

Filming tip: Film B-roll of an actual notepad at a reception desk. It makes the idea instantly visual.

SCRIPT PB-3 · TOFU · ~40 S

Why your ChatGPT scripts sound like every other doctor

Format: Split screen: AI-generated script vs. the same script in a doctor's real voice.

Built from: Content Bank, Pillar 2.2 (script surgery)
Core message: AI can give you a structure, but only you can give it a voice and the right facts.
Value check: Doctors learn how to use AI for scripts without sounding generic or getting facts wrong.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	If your scripts come from ChatGPT, your patients can probably tell.	OST: “Patients can tell.”
3–11s	Here's an AI script: ‘Unlock radiant, glowing skin with our revolutionary treatment.’ Any clinic, anywhere, could post that.	Visual: AI script, highlighted in grey
11–20s	Now a doctor's version: ‘Most of my melasma patients have tried five creams before they see me. Here's why they didn't work.’	Visual: doctor version, highlighted in purple
20–29s	The difference: a real patient situation, your experience, and a reason to keep watching.	OST: “Real patient · Your experience · Open loop”
29–36s	Use AI for structure and ideas. Then add your cases, your opinions and your words, and fact-check every claim.	OST: “AI = structure. You = voice + facts.”
36–40s	Patients don't book a script. They book a doctor.	OST: “They book a doctor.”

CTA: Send me one line from your last script in the comments, and I'll show you how to make it sound like you.

Caption: AI is a great assistant and a terrible spokesperson. Here's how to keep your voice.

Filming tip: Show both scripts as real text on screen, not just spoken. The contrast is the hook.

SCRIPT CB-1 · TOFU · ~40 S

Patients don't choose clinics. They choose doctors.

Format: Talking head + grid comparison: flyer grid vs. doctor-led grid.

Built from: Content Bank, Pillar 5.1 (why clinic flyers don't work)
Core message: Your doctors are your brand. Your flyers are not.
Value check: Clinic owners understand why generic flyers fail and what to replace them with.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	Ninety-nine percent of clinics post the same flyers, and that's exactly why patients can't tell them apart.	OST: “Same flyers. Same clinics.” · Visual: generic flyer grid
3–10s	‘20% off.’ ‘Book now.’ Stock photo, logo, done. Every clinic in the city posts the same thing.	Visual: scroll of flyers
10–18s	But patients don't choose clinics. They choose doctors. A flyer can't answer a nervous patient's question. A doctor on camera can.	OST: “Patients choose DOCTORS”
18–28s	So build your content around your specialists. Each doctor gets a lane: skin, injectables, smiles. And each one answers real patient questions on camera.	Visual: doctor-led grid
28–36s	Now patients arrive already knowing which doctor they want, and why.	OST: “They arrive knowing who they want”
36–40s	Flyers say you have a designer. Your doctors show you have expertise.	OST: “Designer ≠ expertise”

CTA: If you removed your logo, could patients still recognise your clinic's content? Honest answers below.

Caption: Your clinic's real brand is the doctors inside it. Here's how to show them.

Filming tip: Make the before grid from real-looking (fake) flyers you design yourself. Never screenshot another clinic's posts.

SCRIPT CB-2 · MOFU · ~45 S

Your marketing agency isn't medical. Ask them these 5 questions.

Format: Talking head, numbered questions on screen.

Built from: Content Bank, Pillar 5.1 (generic agency = generic content)
Core message: Medical content needs medical review. Most clinic agencies don't have it.
Value check: Clinic owners get 5 questions to judge any agency, including you.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	Clinic owners: if your marketing agency isn't medical, ask them these five questions.	OST: "5 questions for your agency"
3–9s	One: who checks the medical accuracy of what you post for us?	OST: "1. Who fact-checks?"
9–14s	Two: where do your health claims come from? Any sources?	OST: "2. What sources?"
14–20s	Three: do you know the advertising rules for our country and emirate or state?	OST: "3. Know our local rules?"
20–27s	Four: do you report inquiries and bookings, or only likes and views?	OST: "4. Bookings or likes?"
27–33s	Five: who approves claims before they go live, you or our doctors?	OST: "5. Who approves?"
33–45s	If they can't answer three of these, your content might be quietly risking your reputation. Medical content needs a medical eye.	OST: "Medical content needs a medical eye."

CTA: Save this for your next agency meeting.

Caption: Your agency doesn't need to be doctors. But someone medical should check what goes out under your clinic's name.

Filming tip: Keep the tone helpful, not attacking agencies. Doctors respect fairness.

SCRIPT CB-3 · TOFU · ~40 S

The camera-shy doctor in your clinic can still be a star

Format: Talking head + quick examples of each format (voiceover B-roll, Q&A, paired video).

Built from: Content Bank, Pillar 5.2 (doctor personalities inside a clinic)
Core message: Every doctor can be part of the clinic brand. Choose the format that fits their personality.
Value check: Clinic owners get 3 formats to include doctors who hate the camera.

Script

TIME	SAY	ON SCREEN / VISUAL
0–3s	Every clinic has one doctor who refuses to be on camera. Here's how they can still become a face of your brand.	OST: "Camera-shy doctor?"
3–12s	Format one: voiceover plus B-roll. They explain a treatment, we film their hands, the room, the equipment. Face optional.	OST: "1. Voiceover + B-roll"
12–21s	Format two: Q&A. Someone off-camera asks a real patient question and they simply answer it, like in clinic. No script to memorise.	OST: "2. Answer one question"
21–30s	Format three: pair them with a confident colleague. Two doctors discussing a case feels natural, not performed.	OST: "3. Pair them up"
30–40s	Personality doesn't come from performing. It comes from being clear. And quiet, careful doctors are often the ones patients trust most.	OST: "Clear > charismatic"

CTA: Tag the doctor in your clinic who says 'I'm not a camera person' 😄

Caption: Not every doctor wants to be on camera. They can all still be part of your clinic's brand.

Filming tip: The tag CTA spreads the video inside clinics, which is exactly your audience.

SCRIPT FULL · BOFU · ~2 MIN (REEL VERSION ~90 S, ALSO WORKS FOR YOUTUBE/LINKEDIN)

From one Reel to a returning patient: the complete system I build for clinics

Format: Talking head intro → step-by-step screen recordings on a demo clinic (content calendar, script doc, Instagram DM, booking calendar, WhatsApp reminders, review request). A progress bar across the top showing the 10 stages.

Built from: Content Bank, Pillars 1, 2, 3, 7, 8 & 9 + your signature line
Core message: Content gets you the opportunity. Your systems decide what happens to it.
Value check: Clinic owners see the whole patient journey working end to end, and understand exactly what you do.

Script

TIME	SAY	ON SCREEN / VISUAL
0–5s	This is everything that happens between a patient watching your Reel and coming back to your clinic a second time. Most clinics only do the first step.	OST: "Reel → Returning patient: the full system" · Visual: 10-stage progress bar
5–15s	Step one: strategy. We start with the clinic's goals. Which treatments need more patients this quarter? Who's the ideal patient? Then we build content pillars around that.	OST: "1. Strategy" · Visual: pillar board
15–24s	Step two: ideation. We study what's already working for top doctors in the specialty, and turn the clinic's real patient questions into ideas.	OST: "2. Ideation" · Visual: research sheet
24–34s	Step three: scripting. Every script is written for one patient concern, and fact-checked by a doctor before anyone films it.	OST: "3. Scripting + medical review"
34–42s	Step four: the doctor films in batches, two short sessions a month, and we edit and post.	OST: "4. Film · Edit · Post"
42–52s	Step five: a patient watches a video on laser for pigmentation and sends a DM at 10 PM. Within seconds, she gets a reply in the clinic's voice.	OST: "5. DM → reply in seconds" · Visual: DM arriving 10:02 PM, reply 10:02 PM
52–62s	Step six: a few simple questions. Is this your first visit? What's your main concern? Which days suit you? Anything medical goes straight to the team.	OST: "6. Questions (no medical advice)"
62–70s	Step seven: booking. She's offered two real slots and books one, without anyone in the clinic lifting a finger.	OST: "7. Booked" · Visual: calendar confirmation
70–80s	Step eight: reminders. Forty-eight hours before: confirm and prep. Twenty-four hours: reminder with an easy reschedule link. On the day: directions.	OST: "8. Reminders 48h · 24h · day-of"
80–92s	Step nine: after the visit. Day one, an aftercare check-in. Day seven, how's it going? Day thirty, how does she feel about the result? Then, like every patient, she gets a simple link to leave a review.	OST: "9. Follow-up + feedback + review"

92–104s	Step ten: when she's due for her next session, she gets a personal message: her name, her treatment, why now, and two times. And she comes back.	OST: “10. Recall → returning patient” · Visual: 'Hi Sara…' message
104–115s	Content brought her in. The systems made sure she didn't get lost, at any step. That's the whole idea: content gets you the opportunity. Your systems decide what happens to it.	OST: “Content gets you the opportunity. Systems decide what happens to it.”

CTA: If you want this built for your clinic, DM me ‘SYSTEM’ and I'll show you what it would look like for your specialty.

Caption: Most clinics stop at 'post content'. Here's the full journey from a Reel to a returning patient: strategy, scripting, instant replies, booking, reminders, feedback and recall. This is what I build.

Filming tip: Pin this video. It's your best explainer of what you sell. Build the demo with a fictional clinic ('Glow Dermatology') and fictional patient ('Sara'). Never use real patient data.